

SERVING PEOPLE WITH AUTISM SPECTRUM DISORDER

What is Autism

Excerpt taken from NEUROCOSMOPOLITANISM; NICK WALKER'S NOTES ON NEURODIVERSITY,

AUTISM, AND COGNITIVE LIBERTY

Autism is a **genetically-based human neurological variant**. The complex set of interrelated characteristics that distinguish autistic neurology from non-autistic neurology is not fully understood, but current evidence indicates that the central distinction is that **autistic brains are characterized by particularly high levels of synaptic (neural) connectivity and responsiveness**. This tends to make the autistic individual's **subjective experience more intense and chaotic** than that of non-autistic individuals. On both the sensorimotor and cognitive levels, the autistic mind tends to register more information, and the impact of each bit of information tends to be both stronger and less predictable.

Autism is a developmental phenomenon, meaning that **it begins in utero** and has a pervasive influence on development, on multiple levels, throughout the lifespan. **Autism produces distinctive, atypical ways of thinking, moving, interaction, and sensory and cognitive processing**. One way of looking at it is that autistic individuals have a **different neurological "operating system"** than non-autistic individuals.

According to current estimates, **3.2% of the world's population is autistic (1 in 31 children)**. While the number of individuals diagnosed as autistic has increased continually over the past few decades, evidence suggests that this **increase in diagnosis is the result of increased public and professional awareness**, rather than an actual increase in the prevalence of autism.

Despite underlying neurological similarity, **autistic individuals are vastly different from one another**. Some autistic individuals exhibit exceptional cognitive talents and some are intellectually disabled and need constant support. However, in the context of a society designed around the sensory, cognitive, developmental, and social needs of non-autistic individuals, autistic individuals are almost always disabled to some degree — sometimes quite obviously, and sometimes more subtly.

This paper will discuss these conduct issues and provide suggestions for HillTop Job Coaches on how to respond to and support people with ASD.

Common Symptoms of ASD in Adults Include:

- Difficulty interpreting what others are thinking or feeling
- Trouble interpreting facial expressions, body language, or social cues
- Difficulty regulating emotion
- Trouble keeping up a conversation
- Inflection that does not reflect feelings
- Difficulty maintaining the natural give-and-take of a conversation; prone to monologues on a favorite subject
- Tendency to engage in repetitive or routine behaviors
- Only participates in a restricted range of activities
- Strict consistency to having things in a certain order and following daily routines; outbursts when changes occur

- Deep knowledge of one particular topic, such as a certain branch of science or industry

Social Isolation. People with ASD tend to be socially isolated and do not quickly and easily form bonds with new people in their lives. They do not generally like to be touched, to have people too close, or to have a lot of talk and conversation. They may not look directly at people who address them. There can be a lot of anxiety, stress and agitation expressed during the initial meetings and first weeks of supporting them. This is often when extreme forms of self-injurious behavior, aggression and destruction occur. They may also attempt to escape new social demands by eloping. Once routines and social bonds form, they often show much more tolerance and even expect certain people to always be in their lives.

Getting To Know Someone With Autism. New Job Coaches should accompany people the person knows and likes if possible and introduce themselves calmly and gradually. They may drop by the home for short visits before the person actually starts the program; they can eat lunch or spend time with an existing group before taking over, etc. If this is not possible, then the Job Coach should read and learn from people who know the person before trying to support them alone. They should speak directly to the person from a short distance and offer a hand to introduce themselves. They should avoid getting too close, avoid touching them (even with affection), and allow them to offer a hand in return (or not). The Job Coach should not say too much, or talk about the person in the presence of the person.

Going Out into the Community. The Job Coach should be very concrete with directions and about plans for the day. They should not ask too much of the person with ASD at first and try to go along with their routines and compulsions. Initially, seating them in the car should ideally provide for personal space around them. They may be very intolerant of many common locations in the community, and initial settings visited should not be crowded, loud, or sensitive to conduct issues. Places with food and drinks may need to be avoided until their conduct around these items is known. They should not be taken to stores with items they cannot afford, as they may insist on purchasing something before they are willing to leave. Large parks are the safest place to start to build a relationship and long walks help channel nervous energy while people adjust to each other.

Transitions. People with ASD can experience stress and anxiety during the transition from one program to another, from one activity to another and the transfer from one staff to another. Conduct issues may surface during pick-ups and drop-offs, during the end of one activity and the start of another, and during changes in shifts. Not informing them in advance, hurrying them, speaking harshly to them (or around them) makes the situation worse. Transition conduct may diminish over time as the person becomes tolerant of routines, but some people may need a lot of support each day to enter and exit the program. The Job Coach should consider how they could lower the stress level of each transition. This may involve speaking to them in advance, allowing extra time, offering special incentives once the transition is completed (money, snacks, drinks, special trips) and if the person is too agitated, going away and returning to try again later. Other people who make the transitions stressful may need to be educated or avoided.

Communication. People with ASD have unusual ways of expressing themselves as well as difficulty perceiving and understanding complicated social interactions. Many conduct issues have been attributed to their difficulty saying what they want, identifying how they feel, and learning how to respond to and negotiate with other's demands and needs without conduct issues. Some are aphasic (no speech, but they may have "happy" and "anxious" sounds), some echo some of the speech they hear (directly or from the past), some talk excessively and in great detail about topics they obsess on mostly. They may have devices that speak for them, bring and use pictures or symbols to represent things they want, and/or have a repertoire of conduct issues that effectively get them what they want.

The Job Coach will listen to them, encourage them to "use their words", and help make and/or use communication methods that were effective in their past. The Job Coach will make every effort to learn how they communicate and what they are saying and offer prompt and honest responses to each effort to communicate. This may lead to providing what they ask for right away or in the near future plans of the group. Even if the response to the request is a form of refusal ("maybe later", "we already talked about that", "let's ask your home", etc.) acknowledging each

effort to communicate diffuses anxiety over not being heard. Staff insights about the content of each person's communication and how to get them to communicate more should be noted and shared with other people trying to support the person.

Routines and Responding to Chance. Some people with ASD find great comfort in closely following routines to the point of ritualistic adherence to a certain order of things. Others prefer to be told about changes before they occur and need time and lowered expectations when moving through them. Once the HillTop program and a specific Job Coach becomes part of the expected routine of a person with ASD, they often show far less conduct issues, tolerate more, and form strong social bonds. The Job Coach must show consideration to the person they support with ASD about following routines and provide extra attention when there are changes in plans.

Obsessions and Compulsions. People with ASD often have things that they are very focused on. This can be topics of interest, the order of things, drinking certain drinks and/or eating certain foods, the collecting and holding of specific objects, even how they move through space (two steps forward, one step back). It may include how they eat, drink, wash their hands, toilet, how they behave around water, animals, children, etc. They may not tolerate sneezes, florescent lights, or certain sounds. The conduct surrounding these obsessions and compulsions are well developed, often tied to feelings of anxiety, and is well described in their history.

The Job Coach needs to know what the conduct consists of, how it has been accommodated (or controlled) in the past and how they are going to respond to it according to the HillTop behavior plan. This may involve avoidance of certain places and people, the increase of support when certain things are happening, and the careful counseling of the person on what the rules are regarding their conduct in this area.

Over-stimulation. People with ASD often get over-stimulated by sound, light, crowds, touch, too much speech, seeing others experience anxiety, etc. The Job Coach must be aware of what specifically bothers the people they support and the signs that they are not coping well. For example, when a person puts his fingers in his ears he is trying to block sounds that are bothering him. The Job Coach may provide sound-canceling earphones, may leave the setting, or just monitor the person depending upon the potential for conduct issues.

Self-Injurious Behavior (SIB). Some people with ASD express anxiety with highly practiced forms of self-injury. They may have calluses built up from a lifetime of specific bites, hits, banging, etc. Their history of SIB is longstanding and often functions to express and control anxiety, to generate a specific response from caregivers, and to force predictable changes in demands, settings, etc. The Job Coach should learn about the history of the SIB, what situations are likely to trigger it, and what the behavior plan for responding to it is. Often, no comment, intervention, or consequence is directed to the SIB conduct when it occurs. Instead, there will be an effort to gauge and respond to the anxiety that triggered it.

Tolerance and Anxiety. People with ASD have high levels of anxiety and as a result show intolerance of many settings and circumstances similar to where they have had negative past experiences. They need help managing frequent anxiety with proven strategies provided by their Job Coach or else they will use established maladaptive conduct to express and manage it themselves. Proven strategies are provided in behavior plans and often include the following features:

- Effective communication: from the person with ASD and to them by their caregivers. "PLEASE USE YOUR WORDS!" This gives people warning about what is about to happen, provides comfort and coping benefits and provides a functional alternative to severe conduct.
- Relaxation strategies that include deep breathing, music, snacking, moving away, pacing, rocking, etc. should be established and practiced before needed so they can be called upon when the person is stressed.
- Self-control and self-direction strategies need to be in place where the person with ASD needs only request the support they require to get a reliable response from their Job Coach. Saying for example "I need to go!" could lead to the group moving to another place to escape the triggering setting. "I need to talk" could lead to one-on-one support from the Job Coach.
- Changing the setting, allowing them to stop participation, can often allow the person experiencing anxiety to escape the circumstances provoking them. It can be as simple as going to the bathroom or as major as packing up and leaving the setting.
- Offering people options, incorporating their known interests in daily plans and giving them control over their participation in

activities is proven to lessen anxiety and build tolerance.

- Tolerating self-stimming, SIB, acting on obsessions and compulsions and mild forms of target behaviors (but not reinforcing them with attention) may help a person manage their anxiety. They may need to be directed to a setting where they will keep their dignity. This can be tolerated as long as the conduct is not extreme, does not cause harm and is used as a half measure while the person learns more appropriate ways to manage anxiety.
- Providing incentives for good conduct (often defined as the absence of targeted behaviors) provides motivation for self-control. The closer the incentive matches something they care about, the better it will work. How often, the amount offered, and the circumstances where it is provided or denied are specified in a behavior plan. The Job Coach must be consistent, honest and fair when providing an incentive; otherwise it can become a source for increased anxiety.
- Short-term anti-anxiety medications on a PRN basis can avoid major incidents of agitation, aggression and self-endangerment. The medications are administered by the person to themselves when they appear to need it. The Job Coach can point out when it appears necessary and document why it was used and its effect on the person.

Physical Aggression and Property Destruction. Some people with ASD have tremendous histories of becoming severely aggressive and destructive. The expression of this conduct has resulted in one on one support, institutionalization, and demission from programs, high doses of tranquilizing medications, etc. At HillTop, the expression of this conduct is often seen as the breakdown of program methods and not an inevitable part of supporting people with ASD. The Job Coach will do all he/she can to avoid this conduct, and when it occurs, they will carefully describe what triggered it, how it was expressed and how it was resolved. They will participate in the careful analysis of what needs to be done to better support the individual, to maintain safety and whether HillTop is an appropriate placement.